

# “Above the Bias” Chatbot for Abbott Diabetes Care



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# Bias-aware chatbot as strategic extension of Abbott's campaign

- *Above the Bias* campaign highlights misconceptions harming diabetes care
- Support content redundant, fragmented, escalation pathways unclear → undermines trust
- Proposed solution: domain-specific chatbot trained only on Abbott FAQs, product info, campaign content
- Routes users to correct support channel; escalates to customer support or doctor when needed
- Quantified impact: 500K monthly visits → 2% abandonment = \$1.2M annual opportunity cost

There is a common cultural bias that assumes diabetes is self-inflicted. This has real and unintended health consequences for people living with diabetes.

Abbott surveyed over 1,200 people living with diabetes, which revealed that 40% have missed or skipped doctor's appointments due to shame.

A biometric study revealed that they also have a physiological response when exposed to diabetes prejudice.

Hear their stories and support  
**Abbott's commitment to rid the world of diabetes prejudice.**

Visit [www.AboveBias.com](http://www.AboveBias.com) to learn more.

# Use Cases & Scope — Tiered Chatbot Support

Clustering inquiries by frequency and complexity reveals four support tiers

- **Routine FAQs (Tier 0):** Sensor pairing, billing
- **Common Quick Hits (Tier 0/1):** App connection, insurance coverage
- **Rare Escalations (Tier 2):** Unusual error codes, edge cases
- **Critical Pain Points (Tier 1/2 hybrid):** Sensor adhesion failures → chatbot triage → incident report form → support team follow-up
- Chatbot functions when live reps are offline, ensuring 24/7 coverage without replacing human support.

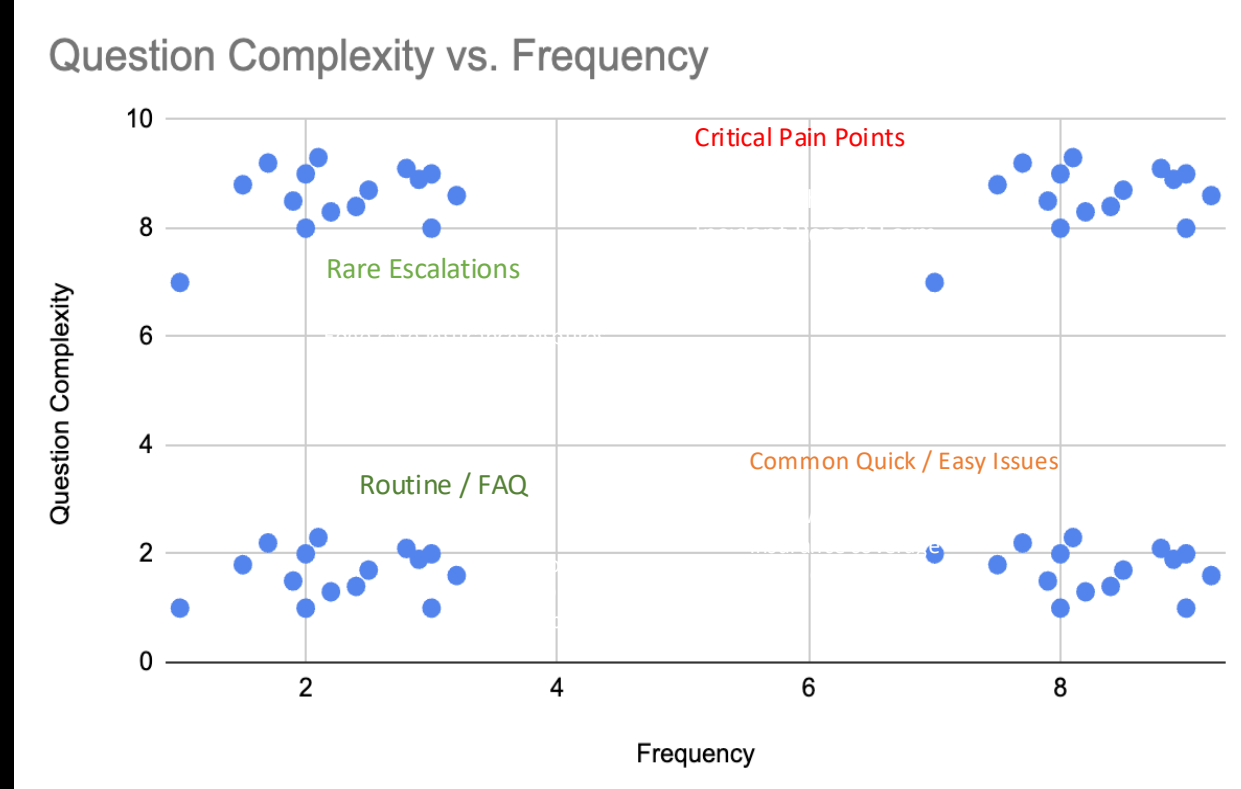


Chart is hypothetical; see "Methodology" section on Slide 6 for details.

*This is **not an emotional support bot**. It is a small language model constrained to Abbott content. For medical or personal questions, it escalates to customer support or a doctor.*

# Architecture: Safe by Design

Retrieval-augmented pipeline with guardrails, explainability, and human oversight

- **Inputs:** FAQs, product info, campaign content
- **Pipeline:** Ingestion → clustering → retrieval → response
- **Controls:** Encryption, audit logs, escalation triggers
- **Human in the loop:** All content validated through **MLR reviews** and subject to **regular testing** with escalation pathways for users to customer support
  - Escalation triggers ensure handoff to live chat when representatives are available.
- **Clarification:** Small language model tuned for Abbott content, not a general LLM

# Operationalize “Above the Bias” through ethical AI governance

Retrieval-augmented pipeline with guardrails, explainability, and human oversight

- **Identify bias:** drift across demographics, languages, devices
- **Mitigate bias:** filter training data, exclude stigmatizing responses, ensure inclusive UX and accessibility
- **Monitor bias:** dashboards, red-flag triggers, escalation alerts
- **Accountability:** model cards, appeal pathways, transparent logs
- **Aligns** directly with *Above the Bias* campaign values

## Change Management & Plan of Action

- **Define scope:** Focus chatbot training on Abbott content, including but not limited to FAQs, product info, and *Above the Bias* campaign content
- **Validate content:** All content must pass **Medical-Legal-Regulatory review**; establish regular testing cycles
- **Build pipeline:** Deploy retrieval-augmented model with clustering + escalation triggers
- **Integrate workflows:** Integrate chatbot with existing live chat system to provide seamless coverage across time zones and availability. Connect chatbot to incident report forms and customer support channels
- **Change management:** Apply LaMarsh overlay to manage adoption, training, and stakeholder buy-in
- **Measure impact:** Track KPIs (CSAT, deflection, bias alerts, resolution time) via dashboards

# METHODOLOGY

- Scatterplot Chart on Slide 3 generated with assistance of CoPilot to create a list of X & Y values to visualize four clusters. Data was imported to Google Sheets to create the chart (<https://docs.google.com/spreadsheets/d/1924rnxUjRtRuRAG03gvpFSUI724tjL1ly7fOpllcD1A/edit?usp=sharing>). Labels were applied in PowerPoint.

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